



ROOT CAUSE UNLOCKED

Unlock Your *Migraines*

Five Keys to Finding What Is Actually Driving Your Migraines

A guide for anyone whose migraines have been managed with medication for years without anyone asking what is setting them off in the first place.

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BEFORE YOU START

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Beyond the Headache

Migraines are far more than just severe headaches. They are a complex neurological condition that can be debilitating, impacting every aspect of a person's life. If you've experienced the throbbing pain, the heightened sensitivity to light and sound, the nausea, and the aura that often precedes an attack, you know the profound disruption migraines can cause. For too long, the conventional approach to migraines has focused primarily on managing symptoms with medication. While these can offer temporary relief, they rarely address the root causes of the problem.

This ebook takes a different path. It is a guide to the interconnected factors that can contribute to migraine development and persistence. I walk through five key areas that are worth investigating when medication alone has not been enough. Think of these as keys: each one opening a door to greater understanding of what is setting your attacks off.

I want to be straight with you about what that means. Migraine is substantially genetic and neurological, and nothing in this book changes the brain you were born with. What the work in these pages can change is how often that brain crosses its threshold. I make no promise here about whether any of it will help you. The measure is your own headache diary compared against your own baseline, which is why I ask you to start one before you change anything.

I walk through the often-overlooked influence of **food sensitivities**, the push and pull of **hormonal balance**, the impact of **chronic infections**, the foundational importance of **structural alignment**, and the role of **nutritional deficiencies**. Alongside each of those I give you what I actually do about it, including dietary strategy, stress management, movement, and the supplements that have real trial evidence behind them.

It's time to shift your perspective from simply enduring migraines to actively investigating them. This approach lets you take an active part in your own care, and it gives you a way of identifying and addressing the underlying drivers that contribute to your migraines. Let's begin.

Food Sensitivities and Allergies · The Hidden Triggers

For many migraine sufferers, the food on their plate can be a powerful, yet often unrecognized, trigger. Unlike immediate, life-threatening food allergies, food sensitivities can manifest hours or even days after consumption, making them challenging to identify. This delayed reaction is why many people don't connect their migraines to specific foods.

Understanding the Difference: Allergy vs. Sensitivity

- **Food Allergies.** These involve an immediate and often severe immune response mediated by IgE antibodies. Symptoms can include hives, swelling, difficulty breathing, and even anaphylaxis. Migraines are rarely the sole symptom of a true IgE-mediated food allergy.
- **Food Sensitivities (Intolerances).** These involve different immune pathways (often IgG, IgA, or IgM antibodies) or non-immune reactions (like enzyme deficiencies). Symptoms are typically delayed and can be varied, including digestive issues, fatigue, skin problems, and, significantly, migraines.

Common Migraine-Triggering Foods

While triggers are highly individual, some foods are frequently implicated in migraine attacks. These often contain compounds that can affect blood vessels, neurotransmitters, or inflammatory pathways.

- **Tyramine-rich foods.** Tyramine is a naturally occurring amino acid that can affect blood pressure and neurotransmitter levels. Found in aged cheeses, cured meats, fermented foods (sauerkraut, kimchi), some red wines, and soy sauce.
- **Nitrates and Nitrites.** Often used as preservatives in processed meats like hot dogs, bacon, and deli meats. These can cause blood vessel dilation.
- **MSG (Monosodium Glutamate).** A flavor enhancer found in many processed foods, Asian cuisine, and some snacks. It can act as an excitatory neurotransmitter.
- **Artificial Sweeteners.** Aspartame and sucralose are commonly cited triggers for some individuals.
- **Caffeine.** While caffeine can sometimes alleviate a migraine in its early stages, regular high intake or sudden withdrawal can trigger headaches.
- **Chocolate.** Contains phenylethylamine and caffeine, both of which can be problematic for some.
- **Alcohol.** Especially red wine, which contains histamines and sulfites, both potential migraine triggers.
- **Citrus Fruits.** For some, the acidity or specific compounds in citrus can be problematic.
- **Gluten and Dairy.** These are two of the most common culprits in food sensitivities, contributing to systemic inflammation and gut dysbiosis, which can then cascade into migraine attacks.

The Gut-Brain Connection and Inflammation

The gut is central to overall health, and its state significantly influences brain function and inflammation. A compromised gut barrier (leaky gut) can allow undigested food particles to enter the bloodstream, triggering an immune response and widespread inflammation. This chronic inflammation can directly contribute to migraine frequency and intensity. Food sensitivities are a major driver of this inflammation.

Identifying Your Triggers: The Elimination Diet

The most effective way to identify food sensitivities is through a carefully executed **elimination diet**. This involves:

1. **Preparation Phase.** For a few days, eat only whole, unprocessed foods. This helps to stabilize your system before the elimination.
2. **Elimination Phase (3 to 4 weeks).** Completely remove common migraine triggers and suspected foods from your diet. This typically includes gluten, dairy, soy, corn, eggs, nuts, nightshades (tomatoes, potatoes, peppers, eggplant), citrus, and any other foods you suspect. During this phase, focus on a diet of lean proteins, a wide variety of vegetables, healthy fats, and gluten-free grains like rice and quinoa. Keep a detailed food and symptom journal.
3. **Reintroduction Phase.** After the elimination phase, slowly reintroduce one food group at a time, every 2 to 3 days. Monitor your symptoms closely. If a food triggers a migraine or other symptoms, it's likely a sensitivity. This methodical process allows you to pinpoint your specific triggers.

Practical Tips for Elimination and Reintroduction

- **Read Labels Diligently.** Hidden ingredients can sabotage your efforts.
- **Cook at Home.** This gives you full control over ingredients.
- **Be Patient.** It takes time to see results and identify triggers.
- **Consult a Professional.** Working with a registered dietitian or a functional medicine practitioner can provide guidance and support throughout this process.

By systematically identifying and avoiding your specific food triggers, you remove one of the inputs that pushes the brain over its threshold. Judge it the way I would in the office, against your own diary over a defined window rather than against anyone's promise.

Hormonal Dysfunction · The Endocrine Orchestra

Hormones are chemical messengers that regulate nearly every process in the body, from metabolism and mood to sleep and reproduction. When this finely tuned system falls out of balance, it can have profound effects, including triggering or exacerbating migraines. Hormonal migraines are particularly common in women, often fluctuating with the menstrual cycle, pregnancy, and menopause, but hormonal imbalances can affect anyone.

Sex Hormones: Estrogen and Progesterone

The most well-known link between hormones and migraines involves **estrogen**. Fluctuations in estrogen levels, rather than consistently high or low levels, are often the culprits.

- **Estrogen Withdrawal Migraines.** Many women experience migraines in the days leading up to or during their period. This is often due to the sharp drop in estrogen that occurs at this time.
- **Perimenopause and Menopause.** The erratic hormonal shifts during perimenopause and the eventual decline in estrogen during menopause can lead to new onset or worsening migraines for some women. Conversely, some women find their migraines improve after menopause when estrogen levels stabilize at a lower baseline.
- **Oral Contraceptives.** For some, birth control pills can alleviate migraines by leveling out hormone fluctuations. For others, they can worsen migraines due to the synthetic hormones.
- **Pregnancy.** Many women experience a significant reduction in migraines during pregnancy due to consistently high estrogen levels. However, some may experience migraines for the first time or find existing migraines worsen.

Progesterone also plays a role. It generally has a calming effect and can counteract some of the excitatory effects of estrogen. An imbalance between estrogen and progesterone (estrogen dominance) can contribute to migraines.

Adrenal Hormones: Cortisol and Stress

The adrenal glands produce hormones like **cortisol**, our primary stress hormone. Chronic stress, whether physical, emotional, or psychological, keeps the adrenals on high alert, leading to elevated cortisol levels.

- **Cortisol Dysregulation.** Sustained stress is proposed to blunt the normal cortisol rhythm over time. “Adrenal fatigue” is the popular name for this and it is not an accepted diagnosis, so I use it as a description of a pattern rather than as a condition. What is not in dispute is that a disordered stress response affects blood sugar stability, inflammation, and sleep, all of which can contribute to migraines.

- **Stress Response.** When we are under stress, our bodies enter a “fight or flight” mode. This can cause blood vessel constriction and then rebound dilation, a common mechanism in migraine development.
- **Sleep Deprivation.** Chronic stress often leads to poor sleep, which is a significant migraine trigger.

Thyroid Hormones: The Body’s Metabolic Regulator

The thyroid gland produces hormones (T3 and T4) that regulate metabolism, energy production, and nerve function. Both **hypothyroidism** (underactive thyroid) and **hyperthyroidism** (overactive thyroid) can be associated with migraines.

- **Hypothyroidism.** Can lead to slower metabolism, fatigue, constipation, and increased inflammation, all factors that can contribute to migraines.
- **Hyperthyroidism.** Can cause anxiety, rapid heart rate, and increased excitability, which may also trigger migraines in susceptible individuals.

Addressing Hormonal Imbalances

Identifying and addressing hormonal imbalances requires a comprehensive approach.

1. **Testing - Sex Hormones.** Saliva or DUTCH (Dried Urine Test for Comprehensive Hormones) testing reports bioavailable hormone levels and their metabolites, which serum does not. Read those as suggestive rather than diagnostic, and interpret them against your cycle and your symptoms rather than as a standalone verdict. - **Adrenal Hormones.** Saliva cortisol testing throughout the day can assess the diurnal rhythm of cortisol production. - **Thyroid Hormones.** Beyond TSH, it’s important to test free T3, free T4, reverse T3, and thyroid antibodies (TPO and TgAb) to get a full picture of thyroid function and check for autoimmunity.
2. **Lifestyle Modifications - Stress Management.** Implement daily stress-reducing practices like meditation, deep breathing exercises, yoga, spending time in nature, or engaging in hobbies. - **Prioritize Sleep.** Aim for 7 to 9 hours of quality sleep per night. Establish a consistent sleep schedule and create a relaxing bedtime routine. - **Regular Exercise.** Helps regulate hormones, reduce stress, and improve sleep. - **Balanced Diet.** Support hormone balance by consuming whole, unprocessed foods, healthy fats (avocado, nuts, seeds, olive oil), and adequate protein. Limit sugar, refined carbohydrates, and inflammatory foods. - **Liver Support.** The liver does much of the work of metabolizing and detoxifying hormones. Support liver health with cruciferous vegetables, milk thistle, and adequate hydration.
3. **Targeted Supplementation - Magnesium.** Critical for hormone regulation and can help reduce the frequency and severity of migraines. - **B Vitamins.** Especially B6, which is involved in estrogen metabolism, and B12 and folate, which support methylation processes important for hormone balance. - **Adaptogenic Herbs.** Ashwagandha, rhodiola, and holy basil can help the body adapt to stress and regulate cortisol. - **Chasteberry (Vitex agnus-castus).** Can help balance estrogen and progesterone, particularly useful for PMS-related migraines. - **DIM (Diindolylmethane).** Found in cruciferous vegetables, DIM supports healthy estrogen metabolism.

4. **Bioidentical hormone replacement therapy (BHRT).** A prescription hormone therapy that some physicians use for perimenopausal and menopausal symptoms, including migraine that tracks with estrogen withdrawal. It is not something I prescribe, and it is included here so you know it exists and can raise it with the physician managing that part of your care. Note also that migraine with aura changes the risk calculation around estrogen-containing therapy, which is a conversation for that prescriber and not for a book.

Addressing hormonal imbalances is a complex but often worthwhile step in migraine work. In my clinical experience, women whose attacks cluster tightly around the menstrual drop are the ones who get the most out of this key, and the way you find out whether you are one of them is by marking cycle days in the same diary where you mark attacks.

Chronic Infection · The Stealth Invaders

While not always immediately apparent, chronic infections can silently undermine health, provoke systemic inflammation, and directly contribute to the onset and persistence of migraines. These are often stealth infections, meaning they can evade the immune system and persist for long periods without causing acute, easily identifiable symptoms.

Understanding the Link

Chronic infections can trigger migraines through several mechanisms:

- **Chronic Inflammation.** Pathogens can stimulate a persistent inflammatory response throughout the body, including the brain. Neuroinflammation is a significant contributor to migraine pathology.
- **Immune System Dysregulation.** A constant battle with an infection can exhaust the immune system, making the body more susceptible to other stressors and less able to cope with existing inflammatory burdens.
- **Toxin Production.** Many pathogens produce neurotoxins that can directly irritate the nervous system and contribute to neurological symptoms, including headaches.
- **Nutrient Depletion.** Chronic infections can deplete the body of essential nutrients needed for nerve function and overall health.
- **Gut Dysbiosis.** Many infections, particularly parasitic or fungal, directly impact gut health, leading to leaky gut and further systemic inflammation (as discussed in Key 1).

Common Chronic Infections Implicated in Migraines

1. **Lyme Disease and Co-infections - Lyme Disease (*Borrelia burgdorferi*).** A tick-borne illness that can become chronic and disseminate throughout the body, affecting the nervous system. Neurological Lyme symptoms are common and can include chronic headaches, brain fog, fatigue, and nerve pain. - **Co-infections (*Bartonella*, *Babesia*, *Ehrlichia*, *Anaplasma*, *Mycoplasma*).** Often transmitted alongside Lyme, these can exacerbate symptoms and complicate treatment. *Bartonella*, in particular, is frequently associated with neurological symptoms and severe headaches.
2. **Mold Illness (Chronic Inflammatory Response Syndrome - CIRS)** - Exposure to water-damaged buildings can lead to the inhalation of mold spores and mycotoxins (toxic compounds produced by molds). - For genetically susceptible individuals, these mycotoxins are not efficiently cleared from the body and trigger a chronic inflammatory response. - Symptoms are diverse and can include debilitating headaches/migraines, fatigue, brain fog, sinus issues, and neurological problems.

3. **Chronic Viral Infections - Epstein-Barr Virus (EBV).** The virus that causes mononucleosis, EBV can remain latent in the body and reactivate under stress or immune compromise. Chronic EBV can contribute to fatigue, neurological symptoms, and systemic inflammation. - **Herpes Simplex Virus (HSV), Cytomegalovirus (CMV).** Other common viruses that can cause chronic issues in some individuals.
4. **Parasitic Infections** - Intestinal parasites can cause chronic inflammation, gut dysbiosis, and nutrient malabsorption, all of which can contribute to migraines. Examples include Giardia, Blastocystis hominis, and various worms.
5. **Small Intestinal Bacterial Overgrowth (SIBO)** - An overgrowth of bacteria in the small intestine can lead to significant digestive issues, malabsorption, and systemic inflammation. The bacterial byproducts and the inflammation itself can trigger migraines.
6. **Chronic Sinus Infections/Fungal Sinusitis** - Persistent inflammation and infection in the sinuses can cause referred pain to the head and contribute to chronic headaches and migraines.

Investigating Chronic Infections

Diagnosing chronic infections can be challenging, as standard tests often miss them. A thorough functional medicine practitioner will consider:

- **Comprehensive History and Symptom Picture.** A detailed history of exposure (tick bites, moldy environments, travel) and a full symptom review are essential.
- **Specialized Lab Testing**
- **Lyme & Co-infections.** Western Blot and ELISA tests may not be sensitive enough. Consider specialized labs like IGeneX or Galaxy Diagnostics for more comprehensive testing (e.g., PCR, FISH, culture).
- **Mold/Mycotoxins.** Urinary mycotoxin panels document recent exposure, including exposure from food, rather than proving that a stored body burden is driving your symptoms. Read them as suggestive rather than diagnostic. Environmental testing of the building (ERMI, HERTSMI-2) assesses the place you live, and the strongest evidence in this whole area is for fixing the building.
- **Viral Panels.** Comprehensive viral antibody panels to assess for active or reactivated infections (e.g., EBV early antigen, CMV IgA/IgG/IgM).
- **SIBO.** Hydrogen and methane breath tests.
- **Parasites.** Comprehensive stool analysis with PCR testing to identify parasites and imbalances in gut flora.

Treatment Strategies

Treatment for chronic infections is highly individualized and depends on the specific pathogen identified. It often involves a multi-pronged approach:

1. **Antimicrobial Therapies - Prescription antimicrobials.** Antibiotics, antifungals, and antiparasitics are prescription drug classes that some physicians use for documented infections. They are not something I prescribe, and they are included here so you know they exist and can raise them with the physician managing that part of your care. - **Herbal Antimicrobials.** Many herbs have antimicrobial activity in the laboratory (e.g., Cat's Claw, Andrographis, Japanese Knotweed for Lyme; Oregano oil, Berberine for SIBO; Pau d'arco, Garlic for Candida). Laboratory activity is not the same as a human outcome, and in most of these the human outcome trials have never been funded. I still use several of them, sequenced and with an endpoint, and that is what I am asking you to buy: reasoned clinical practice, not proven certainty. Oregano oil belongs at a cap of one to two capsules total per day.
2. **Detoxification Support** - Binding agents (e.g., activated charcoal, bentonite clay, chlorella) to help eliminate toxins. **Take a binder 1 to 2 hours away from food, supplements, and medication**, because a binder does not know the difference between a toxin and your thyroid tablet. - Liver support (e.g., milk thistle, N-acetylcysteine - NAC, glutathione). - Infrared sauna and lymphatic drainage to aid drainage pathways. Nobody has funded a human trial on whether sweating clears these compounds, which tells you more about research funding than about your body. I use sauna as a supportive measure while the exposure fix does the heavy lifting. Hydrate, replace electrolytes, avoid it in pregnancy, and clear it with a clinician if you have cardiovascular disease.
3. **Immune System Modulation** - Nutrients like Vitamin D, Vitamin C, Zinc, and Selenium. - Herbs like medicinal mushrooms (Reishi, Cordyceps), Astragalus.
4. **Gut Health Restoration** - Probiotics and prebiotics to rebalance the microbiome. - Gut-healing nutrients (e.g., L-glutamine, collagen, bone broth).
5. **Environmental Remediation** - Essential for mold illness: Professional mold remediation of water-damaged buildings.

Addressing chronic infections is long, difficult work, and here is where the evidence sits. The association between these organisms and migraine is largely observational, and no trial has shown that treating them reduces migraine days. That is a statement about what has been studied, not a verdict on the tools. Here is what I do with it clinically. I investigate this key when the picture demands it, meaning a clear exposure history, symptoms well beyond the head, or a migraine pattern that has not budged after the diet, sleep, and nutrient work. I do not open this door first, because it is the most expensive and slowest of the five. Working with a doctor experienced in chronic infectious disease or functional medicine matters here more than anywhere else in this book.

Structural Imbalance · The Body's Foundation

Our bodies are complex biomechanical structures. Structural imbalance creates sustained muscular tension and altered sensory input from the neck and jaw, and both of those can feed a migraine. This key focuses on how issues in your musculoskeletal system reach your head. It is also the key where the marketing runs furthest ahead of the evidence, so I have marked the difference as I go.

The Spine: Your Central Support

The **cervical spine (neck)** is where I spend most of my hands-on time with migraine patients, and I want to separate what is well established from what is argued.

- **Nerve Irritation, the part that is well established.** The trigeminal nerve, a major player in migraine, converges with sensory input from the upper cervical nerve roots. That convergence is standard neuroanatomy and it is the reason neck input can produce or amplify head pain, and the reason a genuine neck-driven headache can be almost indistinguishable from migraine at the bedside. This is the mechanism I actually work on.
- **Blood flow, a claim I am not going to assert.** You will read that upper cervical misalignment restricts the vertebral arteries and starves the brainstem. That is a proposed mechanism, it is still argued in the literature, and it has not been demonstrated to drive migraine. I do not use it as a rationale for treatment, and I would be cautious with anyone who uses it that way.
- **Muscle Tension.** Chronic tension in the neck and shoulder muscles (e.g., trapezius, sternocleidomastoid, suboccipitals) can refer pain to the head, and in my clinical experience it is one of the most reliably modifiable inputs in the whole structural picture.

Myofascial Tension: The Web of Connective Tissue

Myofascia is the tough, elastic connective tissue that surrounds muscles, bones, and organs. When this tissue becomes tight, restricted, or develops “trigger points” (hyperirritable spots in muscles), it can create widespread pain and dysfunction.

- **Trigger Points.** Specific trigger points in the neck, shoulders, and even jaw muscles can refer pain directly to the head, mimicking or triggering migraines. For example, a trigger point in the sternocleidomastoid muscle can cause pain behind the eye.
- **Fascial Restrictions.** Restrictions in the fascia can limit movement, alter posture, and create chronic tension patterns that perpetuate migraine cycles.

Postural Alterations: The Cumulative Effect

Modern lifestyles often lead to poor posture, such as “forward head posture” from prolonged computer use or smartphone gazing.

- **Increased Strain.** For every inch your head moves forward, the weight on your cervical spine significantly increases. This puts enormous strain on the neck muscles and discs, leading to chronic tension and potential nerve compression.
- **Breathing Impairment.** Poor posture can restrict diaphragmatic breathing, leading to shallow chest breathing, which can increase stress and perpetuate muscle tension.

TMJ Dysfunction: The Jaw Connection

The temporomandibular joint (TMJ), which connects your jaw to your skull, is another often-overlooked area.

- **Referred Pain.** Dysfunction in the TMJ (e.g., teeth grinding, clenching, misalignment) can cause referred pain to the temples, eyes, and ears, often mistaken for or contributing to migraines.
- **Muscle Hyperactivity.** Overactive jaw muscles can create widespread facial and head tension.

Addressing Structural Imbalances

Correcting structural imbalances requires a multi-faceted approach, often involving manual therapies and active rehabilitation.

1. **Chiropractic Care - Upper Cervical Chiropractic.** Focuses on the upper cervical segments, the atlas (C1) and axis (C2). The proposed mechanism is a change in segmental joint function and in the sensory input arriving from that region, which is a reasonable target given the trigeminocervical convergence described above. What it is not is a demonstrated correction of nerve flow or blood supply, and I do not describe it that way to patients. - **Full Spine Chiropractic.** Addresses misalignments throughout the spine, improving overall posture and reducing compensatory patterns.
2. **Osteopathic Manipulative Treatment (OMT)** - Osteopathic physicians use a variety of hands-on techniques to diagnose and treat musculoskeletal imbalances, improve circulation, and reduce somatic dysfunction.
3. **Physical Therapy** - Focuses on strengthening weak muscles, stretching tight ones, improving posture, and restoring proper movement patterns. - Includes exercises for neck stability, shoulder mobility, and core strength.
4. **Myofascial Release and Massage Therapy - Myofascial Release.** Specific techniques to release restrictions in the fascia. This can be done by a trained therapist or with tools like foam rollers and lacrosse balls. - **Deep Tissue Massage.** Helps release muscle knots and reduce tension in the neck, shoulders, and back.

5. **Acupuncture** - Can help release muscle tension, reduce pain, and improve circulation by stimulating specific acupuncture points.
6. **Craniosacral Therapy (CST)** - A gentle, hands-on method of evaluating and enhancing the functioning of the craniosacral system (membranes and cerebrospinal fluid surrounding the brain and spinal cord). It aims to release restrictions that can affect neurological function.
7. **Ergonomics and Posture Correction** - Evaluate your workstation, car seat, and sleeping position. Ensure proper support and alignment. - Practice conscious posture awareness throughout the day. - Take frequent breaks to move and stretch, especially if you have a sedentary job.
8. **TMJ Specialist/Dentist** - If TMJ dysfunction is suspected, consult a dentist specializing in TMJ or an orofacial pain specialist. Treatments may include splints, bite adjustments, or physical therapy for the jaw.

By addressing the underlying structural issues, you can alleviate the physical tension and nerve irritation that often contribute to migraine attacks. That puts another essential piece of the migraine puzzle in place.

Nutritional Deficiencies · Fueling Your Brain

Our brains are incredibly complex organs that require a constant supply of specific nutrients to function optimally. Just as a car needs the right type of fuel and oil, your brain needs adequate vitamins, minerals, healthy fats, and amino acids. Deficiencies in these critical building blocks can impair neurological function, increase inflammation, and lower your migraine threshold.

What you are buying here, stated plainly. This is the one key in the book where the evidence is genuinely good, and it is worth knowing that before you spend anywhere else. Magnesium, riboflavin, and CoQ10 have been through randomized controlled trials in migraine prophylaxis, and the doses that worked in those trials are specific. Vitamin D and omega-3 have trial and meta-analytic evidence that is real but more mixed. The rest of what appears in a typical migraine supplement stack, including the adaptogens, chasteberry, and DIM further up in this book, has laboratory work and clinical use behind it rather than migraine outcome trials, and that is what happens to compounds nobody can patent: the trials never get funded. So put your money in the order the evidence sits, hold anything you start to a defined endpoint, and give each one enough time to be judged. The companion supplement guide in this kit lays out that evidence search in full, including what I could not find.

Essential Nutrients for Migraine Prevention

1. **Magnesium - Role.** Involved in over 300 enzymatic reactions, including nerve transmission, muscle relaxation, blood vessel tone, and energy production. Many migraineurs are magnesium deficient. - **How it helps migraines.** Helps regulate neurotransmitters, reduces neuromuscular excitability, and dilates blood vessels, which can be constricted during migraines. - **Food Sources.** Dark leafy greens, nuts (almonds, cashews), seeds (pumpkin, chia), legumes, whole grains, avocado, dark chocolate. - **Supplementation.** Magnesium prophylaxis has been tested in randomized trials in migraine without aura.^{1,2} Form and dose are in the recap at the end of this book.
2. **Riboflavin (Vitamin B2) - Role.** A key component of coenzymes involved in energy production (ATP) within cells. - **How it helps migraines.** Improves mitochondrial function, which can be impaired in migraineurs, leading to better cellular energy and reduced susceptibility to attacks. - **Food Sources.** Liver, beef, lamb, fortified cereals, mushrooms, spinach, almonds. - **Supplementation.** High-dose riboflavin for migraine prophylaxis was tested in a randomized controlled trial.³ Dose is in the recap at the end of this book, and it is the specific dose that matters here.
3. **Coenzyme Q10 (CoQ10) - Role.** A powerful antioxidant and essential for mitochondrial energy production. - **How it helps migraines.** Improves cellular energy, reduces oxidative stress, and may stabilize vascular function. - **Food Sources.** Organ meats (heart, liver, kidney), beef, sardines, mackerel, spinach, broccoli, peanuts. - **Supplementation.** CoQ10 in migraine prophylaxis was tested in a randomized controlled trial.⁴ Form and dose are in the recap at the end of this book.

4. **Vitamin D - Role.** More than just a vitamin, it acts like a hormone, influencing immune function, inflammation, and neurological health. - **How it helps migraines.** Anti-inflammatory properties, modulates immune response, and may influence neurotransmitter pathways. Low levels are common in migraine sufferers. - **Food Sources.** Fatty fish (salmon, mackerel, tuna), fortified dairy products, some mushrooms (if exposed to UV light). Primary source is sunlight exposure. - **Supplementation.** The effect of vitamin D on migraine has been examined in a meta-analysis.⁵ Test your level rather than guessing at a dose, and see the recap at the end of this book.
5. **Omega-3 Fatty Acids (EPA & DHA) - Role.** Essential fats with potent anti-inflammatory properties, important for brain health and cell membrane integrity. - **How it helps migraines.** Reduce systemic inflammation, modulate pain pathways, and improve vascular health. - **Food Sources.** Fatty fish (salmon, sardines, anchovies, mackerel), flaxseeds, chia seeds, walnuts. - **Supplementation.** Dietary alteration of n-3 and n-6 fatty acids for headache reduction in adults with migraine was tested in a randomized controlled trial.⁶ Note what that trial changed: the diet, not only a capsule. Dose guidance is in the recap at the end of this book.
6. **B Vitamins (B6, B9/Folate, B12) - Role.** Critical for methylation, a fundamental biochemical process involved in neurotransmitter synthesis, detoxification, and DNA repair. - **How it helps migraines.** They support neurological function and they are the inputs that keep homocysteine down. Homocysteine deserves more than a passing mention. It is a methylation-cycle marker. It rises when B12, folate, or B6 run short, and with thyroid or kidney trouble. Do not let anyone wave off an elevated result as trivia. The way to act on it is not to chase the number with blind supplementation. Find which input is short, B12, folate, B6, thyroid, or kidney function, correct that, and let homocysteine confirm the fix on retest. - **Food Sources**
- **B6.** Poultry, fish, potatoes, bananas, chickpeas.
 - **Folate.** Leafy greens, legumes, liver, asparagus, broccoli.
 - **B12.** Meat, fish, dairy, eggs (found primarily in animal products).
 - **Supplementation.** Dietary folate intake has been examined against migraine disability and frequency.⁷ That is dietary intake rather than a high-dose supplement trial, which is worth knowing before you buy anything. Form guidance is in the recap at the end of this book.

Comprehensive Nutritional Strategies

1. **Whole Foods Diet - Prioritize.** A wide variety of organic, unprocessed fruits, vegetables, lean proteins, healthy fats, and gluten-free whole grains. - **Limit/Avoid.** Processed foods, refined sugars, artificial additives, inflammatory oils (canola, soy, corn oil), and trans fats.
2. **Hydration** - Dehydration is a common migraine trigger. Aim for at least 8 glasses of filtered water daily, more if exercising or in hot climates.
3. **Blood Sugar Balance** - Fluctuations in blood sugar can trigger migraines. Eat regular, balanced meals containing protein, healthy fats, and complex carbohydrates to prevent spikes and crashes.

4. **Gut Health Support** - A healthy gut microbiome is essential for nutrient absorption and overall health. Incorporate fermented foods (kimchi, sauerkraut, kefir) and consider a high-quality probiotic.
5. **Personalized Supplementation** - It's important to identify specific deficiencies through blood tests (e.g., Vitamin D, B12, Magnesium red blood cell levels, Omega-3 index) before supplementing. - Always choose high-quality, bioavailable forms of supplements from reputable brands. - **Consult a Healthcare Professional.** Before starting any new supplement regimen, especially if you are on medication, consult with your doctor or a qualified nutritionist to ensure safety and appropriate dosing.

By optimizing your nutritional intake and addressing any deficiencies, you provide your body and brain with the essential fuel needed to function optimally, reduce inflammation, and build resilience against migraine attacks.

The Unlocking Plan: Integrating the Keys

Now that I have walked you through the five keys, it's time to put them all together. A complete approach recognizes that these keys are interconnected. An inflammatory food sensitivity might exacerbate a hormonal imbalance, which then depletes magnesium, making you more susceptible to structural tension that triggers a migraine. Addressing one key often positively impacts the others.

This section provides a practical framework for integrating these strategies into your daily life.

The Anti-Migraine Diet Plan

This diet forms the foundation, addressing potential food sensitivities and providing essential nutrients.

General Guidelines:

- **Eliminate Common Triggers (initially).** Gluten, dairy, soy, corn, eggs, nightshades (tomatoes, potatoes, peppers, eggplant), citrus, peanuts, and any other foods identified through your elimination diet. Gradually reintroduce to test.
- **Focus on Whole, Unprocessed Foods.** Prioritize organic whenever possible.
- **Balance Macronutrients.** Every meal should include protein, healthy fats, and complex carbohydrates (from vegetables and gluten-free grains).
- **Hydrate Adequately.** Drink at least 8 to 10 glasses of filtered water daily.
- **Avoid Artificial Additives.** Steer clear of artificial sweeteners, food dyes, and MSG.
- **Limit Sugar and Refined Carbohydrates.** These can cause blood sugar spikes and crashes, contributing to inflammation.

Sample Daily Meal Plan:

- **Breakfast**
- **Option 1.** Green smoothie with spinach, kale, cucumber, water, protein powder (collagen or pea protein), 1/2 banana, and chia seeds.
- **Option 2.** Scrambled eggs (if not sensitive) with sauteed spinach and mushrooms, served with 1/2 avocado. (If sensitive to eggs, replace with lean ground turkey or a plant-based scramble.)
- **Lunch**
- Large salad with mixed greens, grilled chicken or fish (salmon, cod), various non-starchy vegetables (cucumber, carrots, bell peppers if not sensitive), and a dressing of olive oil and apple cider vinegar.
- Alternatively, a bowl of lentil or vegetable soup (homemade with bone broth if possible).
- **Dinner**

- Baked salmon or lean grass-fed beef with roasted broccoli and sweet potato.
- Stir-fry with lean protein (chicken, turkey) and a variety of colorful vegetables (broccoli, carrots, snap peas), using tamari (gluten-free soy sauce alternative) or coconut aminos, and ginger. Serve with quinoa or brown rice.
- **Snacks (as needed)**
- Handful of almonds or walnuts (if not sensitive).
- Apple slices with almond butter.
- Vegetable sticks (celery, carrots, cucumber) with hummus (if legumes are tolerated).
- Small portion of berries.

Lifestyle Management for Stress, Sleep, and Environment

These aspects are critical for hormone balance, immune function, and overall resilience.

1. **Stress Management - Mindfulness & Meditation.** Even 10 to 15 minutes daily can significantly reduce stress response. Use apps like Calm or Headspace. - **Deep Breathing Exercises.** Calms the nervous system. Practice diaphragmatic breathing several times a day. - **Yoga/Tai Chi.** Gentle movement combined with breathwork. - **Nature Immersion.** Spend time outdoors; “forest bathing” has proven stress-reducing benefits. - **Hobbies & Creative Outlets.** Engage in activities you enjoy; they add joy and reduce stress. - **Boundaries.** Learn to say no and protect your time and energy. - **Therapy/Counseling.** If stress is overwhelming, seek professional support.
2. **Sleep Optimization - Consistent Sleep Schedule.** Go to bed and wake up at the same time every day, even on weekends. - **Dark, Cool, Quiet Room.** Optimize your sleep environment. Blackout curtains, earplugs, and a comfortable temperature are key. - **Limit Screen Time.** Avoid screens (phones, tablets, computers, TV) at least 1 to 2 hours before bed. - **Caffeine & Alcohol.** Limit consumption, especially in the afternoon and evening. - **Wind-Down Routine.** Take a warm bath, read a book, or listen to calming music.
3. **Environmental Toxin Reduction - Indoor Air Quality.** Use air purifiers, open windows for ventilation, address any mold issues promptly and professionally. - **Water Quality.** Invest in a good water filter for drinking and showering. - **Personal Care Products.** Choose non-toxic, fragrance-free products. - **Cleaning Supplies.** Opt for natural, chemical-free cleaners. - **Pesticides & Herbicides.** Avoid exposure in your home and garden.

Exercise: Movement for Migraine Relief

Regular, moderate exercise can significantly help reduce migraine frequency and severity, but it's important to find the right balance. Overexertion can be a trigger for some.

- **Aerobic Exercise (Moderate Intensity)**

- **Examples.** Brisk walking, swimming, cycling, elliptical.
- **Frequency.** Aim for 30 minutes, 3 to 5 times per week.
- **Benefits.** Improves circulation, reduces stress, releases endorphins (natural pain relievers), and helps regulate sleep.
- **Strength Training**
- **Examples.** Bodyweight exercises, light weights, resistance bands.
- **Frequency.** 2 to 3 times per week.
- **Benefits.** Improves posture, strengthens core and neck muscles, which can reduce structural imbalances.
- **Flexibility and Mobility**
- **Examples.** Yoga, Pilates, stretching.
- **Frequency.** Daily or several times per week.
- **Benefits.** Reduces muscle tension, improves range of motion, and enhances body awareness, addressing myofascial and structural issues.
- **Listen to Your Body.** If a particular exercise aggravates your migraines, modify it or choose a different activity. Start slowly and gradually increase intensity and duration.

At-Home Remedies and Acute Management

While the focus is on prevention, having strategies for acute migraine management is essential.

- **Dark, Quiet Room.** Retreat to a calm environment to reduce sensory overload.
- **Cold Compress.** Apply a cold pack to the forehead, temples, or back of the neck to constrict blood vessels and numb the area.
- **Heat Therapy.** A warm shower or heating pad on the neck and shoulders can help relax tense muscles.
- **Caffeine (Carefully).** A small amount of caffeine early in an attack can sometimes help, but be cautious of rebound headaches from overuse.
- **Essential Oils**
- **Peppermint Oil.** Dilute and apply to temples and forehead for a cooling, pain-relieving effect.
- **Lavender Oil.** Inhale or diffuse for relaxation and pain relief.
- **Magnesium Bath/Foot Soak.** Dissolve Epsom salts (magnesium sulfate) in warm water for a relaxing soak, allowing transdermal absorption of magnesium.
- **Hydration.** Sip on water or an electrolyte solution.
- **Gentle Stretching.** Light neck and shoulder stretches if they don't worsen the pain.
- **Acupressure.** Press firmly on points like the web between the thumb and forefinger (LI4) or the temples.

Key Nutritional Supplements (Recap with Dosing Guidance - *Always consult a healthcare professional*)

- **Magnesium**
 - **Form.** Magnesium glycinate, malate, or citrate (highly absorbable). Avoid magnesium oxide as it's poorly absorbed.
 - **Dose.** 400 to 600 mg daily, divided doses to avoid digestive upset. Start lower and gradually increase.
- **Riboflavin (Vitamin B2)**
 - **Form.** Riboflavin.
 - **Dose.** 400 mg daily. Effects may take several weeks.
- **Coenzyme Q10 (CoQ10)**
 - **Form.** Ubiquinol (more bioavailable).
 - **Dose.** 100 to 300 mg daily.
- **Vitamin D3**
 - **Form.** Vitamin D3 (cholecalciferol).
 - **Dose.** Varies significantly based on current levels and sun exposure. Often 2,000 to 5,000 IU daily, but guided by blood tests. Take with Vitamin K2 for optimal absorption and calcium regulation.
- **Omega-3 Fatty Acids**
 - **Form.** High-quality fish oil with high EPA/DHA content.
 - **Dose.** 1,000 to 3,000 mg combined EPA+DHA daily.
- **B Complex (Methylated forms)**
 - **Form.** Look for methylated folate (L-5-MTHF) and methylcobalamin (B12).
 - **Dose.** Follow label instructions, typically a balanced B-complex supplement.
- **Other Potential Supplements (discussed in previous sections)**
 - **DIM.** For estrogen metabolism (if relevant).
 - **Adaptogens (Ashwagandha, Rhodiola).** For stress and adrenal support.
 - **Chasteberry.** For PMS-related migraines.
 - **Probiotics.** For gut health.

Important Note. The information provided on supplements is for educational purposes only. It is **crucial** to consult with a qualified healthcare professional (doctor, registered dietitian, functional medicine practitioner) before starting any new supplement regimen, especially if you have underlying health conditions or are taking medications. They can help determine appropriate dosages and ensure there are no contraindications or interactions.

What to Do With All of This

Working out what drives your migraines takes investigation, commitment, and persistent effort rather than a quick fix, and it is about moving beyond simply managing symptoms to understanding what is setting the attacks off. By systematically working through the five keys (food sensitivities, hormonal dysfunction, chronic infections, structural imbalances, and nutritional deficiencies), you get a framework for that investigation.

This approach equips you to:

- **Become a detective of your own health.** Learning to identify your own triggers and the drivers behind your migraines.
- **Take an active role in your care.** Shifting from passive patient to engaged participant.
- **Judge every change against a real baseline.** Your headache diary before you started, compared to your headache diary now. That is the only measurement in this book that belongs to you rather than to somebody selling something.
- **Improve your overall health.** As you address these drivers, energy, mood, digestion, and general vitality often move with them.

Progress will not be linear. There will be good days and challenging days, and migraine frequency swings on its own, which is exactly why the diary and the one-change-at-a-time discipline matter so much. Be patient with yourself and stay consistent. Finding a healthcare team that supports this approach (a functional medicine doctor, a chiropractor, a nutritionist, a physical therapist) is worth the effort.

I will not tell you where this ends, because I do not know your brain and I have not examined you. What I can tell you is what I have seen clinically: the people who get the most out of this work are the ones who changed one thing at a time, kept the log honestly, and gave each change long enough to be judged. Start there.

Working with Root Health

Book a free Root Discovery Session: rootcauseunlocked.com/discovery, or call the Garner, NC office at (919) 662-0520.

Consultations and ongoing care are both available wherever you are located. Being outside North Carolina is not a barrier to working with me. Tell me where you are when you reach out and I will lay out exactly how we would work together from there.

If you have been told your scans are clear and handed another prescription without anyone asking what sets your attacks off, bring all of it to the session, including your diary if you have started one. I will tell you honestly what I would investigate first and in what order. That conversation is free, and it is the fastest way to stop guessing.

If you would rather start on your own, begin the diary tonight and read the lab guide in this kit before you spend money on testing.

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